

Dysphagia

KEYWORDS / TYPICAL PRESENTATION

- **Difficulty initiating swallow, coughing, choking**, nasal regurgitation: Think **Oropharyngeal Dysphagia**.
 - Key causes: Stroke, Parkinson's, **Zenker's Diverticulum** (which presents with halitosis and regurgitation of undigested food).
- Food "getting stuck" after swallowing: Think **Esophageal Dysphagia**.
- Esophageal dysphagia to **solids only**: Think Mechanical Obstruction (e.g., peptic stricture, Schatzki ring, EoE, tumor).
- Esophageal dysphagia to **solids AND liquids**: Think Motility Disorder (e.g., achalasia, spasm).

DIAGNOSTIC PATHWAY

- **Oropharyngeal Dysphagia**: Best Initial Test is a **Video Swallow Study**.
- **Esophageal Dysphagia**: Best Initial Test is **Upper Endoscopy (EGD)** to rule out mechanical obstruction.
- **Next Step if EGD is non-diagnostic**: Perform Barium Esophagram, followed by Esophageal Manometry (Gold Standard for motility disorders)

Achalasia

KEYWORDS / TYPICAL PRESENTATION

- Dysphagia to solids and liquids, regurgitation of undigested food, chest pain, weight loss.
- **Rapid onset (>60 yo, significant weight loss)**: Must rule out **pseudoachalasia** from GE junction tumor with EGD.

DIAGNOSTIC PATHWAY

- **Barium Esophagram shows**: Dilated esophagus with distal tapering ("bird's beak").
- **Most Accurate Test: Esophageal Manometry** shows absent peristalsis and incomplete LES relaxation.

Core Principles for Achalasia Management:

1. Goal is to disrupt or relax the lower esophageal sphincter (LES).
2. Treatment options include: Pneumatic dilation, surgical myotomy (Heller myotomy), or botulinum toxin injection.
3. Fact: Patients have a long-term increased risk of **esophageal squamous cell carcinoma**.